



AI PRODUCT CASE STUDY

MyCCAngel

Building an AI companion for people living with Crohn's & Colitis — one product decision at a time.

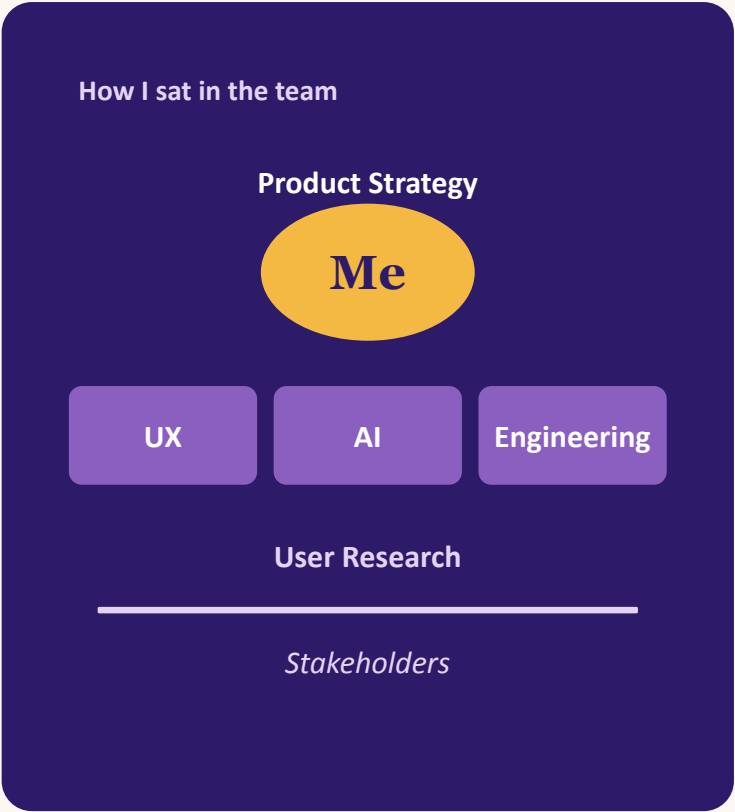
Founding Product Lead · AI · Healthcare · UX

My Role

Founding Product Lead

End-to-end owner: from user problem to shipped experience.

Product vision	Product strategy	User research
Requirements & specs	UX collaboration	Database schema design
AI product design	ML model development	Recommendation engine
Frontend coordination	Backend coordination	Sprint planning



It started with a question

"Why doesn't Crohn's & Colitis have a digital companion — the way diabetes and mental health do?"



That question became a two-year exploration into research, design, ML, and the messy reality of building an AI product in healthcare.

The reality

7M+

people worldwide live with IBD.

Zero

trusted, condition-specific AI companions when I started.

Meet Sara

Sara, 29

*Diagnosed with ulcerative colitis at 24.
Marketing manager. Loves travel, dreads
flare-ups.*

GOAL

Feel in control between doctor visits.

FRUSTRATION

Generic health apps don't understand IBD.

TRUST FILTER

"Show me it's medically real, or I'm out."

What she needs



A companion

Not another symptom tracker gathering dust.



Personalization

Content that adapts to her specific condition & phase.



Trust

Medically-verified information, not random forums.



Everyday utility

Toilet locator, flare warnings, community — real help.

IBD patients live between two worlds

The clinic

15 minutes every 3–6 months. Bloodwork, colonoscopy, prescription. Then silence.

Real life

Flares at 2 a.m. Bathroom anxiety on flights. Confusing forums. Google spirals.

The gap between these two worlds is where quality of life is lost — and where a product can genuinely help.

I stopped guessing and started listening

As Product Lead I ran the discovery: recruited participants, wrote the interview guide, ran the sessions, synthesized themes.

“

"I've been to five doctors. I still don't know which foods trigger me."

— Long-term patient

“

"Every app treats me like I have generic IBS. My case is different."

— Newly diagnosed

“

"I need to know where the nearest bathroom is before I say yes to plans."

— Frequent traveler

Signal: patients don't want another tracker. They want a companion that already understands them.

From tracker → companion.

Most IBD apps

Log symptoms. Show charts. Wait for you to interpret.

My bet: MyCCAngel

Ask the right questions. Verify the medical picture.
Recommend what helps next.

This reframing drove every product decision that followed.

Every feature answered a hard question

As Product Lead, my job wasn't to build everything — it was to decide what to build first, and why.

01	Why verify medical reports?	→	Trust	Without medical proof, personalization is guessing. Verification unlocks credibility.
02	Why start with recommendations?	→	Cold start	New users need value on day one, before any behavior data exists.
03	Why a toilet locator?	→	Highest daily value	The #1 anxiety patients named. Small feature, enormous emotional payoff.
04	Why no doctor directory yet?	→	Needs partnerships	Directories are only useful when curated. Deferred until we had clinical partners.
05	Why AI chatbot in phase 2?	→	Needed trusted content	A chatbot without verified content just amplifies misinformation.

The roadmap: earn trust before scale

Phase 1 • Build trust

Ship the smallest thing that makes users feel understood.

- ✓ Registration
- ✓ Medical verification
- ✓ Profiles
- ✓ Survey
- ✓ Recommendations
- ✓ Messaging
- ✓ AI chatbot
- ✓ Toilet locator

Phase 2 • Grow the ecosystem

Only add breadth once the core loop is trusted.

- ✓ Doctors directory
- ✓ Dietitians
- ✓ Groups & community
- ✓ Events
- ✓ Restaurant finder

A 20-question survey that feels like 6

I designed a branching onboarding survey so the app only asks what's clinically relevant to that user.

1

One question per screen

Removes cognitive load. Feels like a conversation, not a form.

2

Branch by condition state

Symptomatic users get triage. Remission users get maintenance.

3

Every answer earns a feature

No throwaway questions. Each response feeds ML or personalization.

6–10

questions per user

3

branch paths

1

question per screen

The survey, in the user's hands

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How much does IBD impact your life?

Not at all Quite a bit

A little Severely

Moderately

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Which aspect of your daily life has IBD impacted the most?

Physical Health Sexual Life

Mental Health Emotional Connection

Diet & Nutrition Personal Relationships

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Are you currently in Remission or Flare-up?

Remission Asymptomatic evidence

Flare-up (Chronic) Very much Symptomatic

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How long ago did you get this flare-up confirmed or started to become symptomatic?

1-3 Months 1-3 Years

3-6 Months 3-5 Years

6-12 Months More than 5 Years

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How long have you been into remission?

1-3 Months 1-3 Years

3-6 Months 3-5 Years

6-12 Months More than 5 Years

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Do you face any of the below symptoms?

Blood in stool all the time Weakness

Mucus Low on energy always

Sharp abdominal pain Fever (sometimes)

Fever more often

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When did you get the tests done?

Yesterday Two Weeks Ago

This Week Three Weeks Ago

A Week Ago A Month Ago or More

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How many times, on average, have you been to the toilet every day this week?

3-5 times a week 8-15 times a day

1-3 times a day More than 15 times a day

3-8 times a day

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Same visual language across every question — the survey feels like one continuous conversation.

Turning messy PDFs into trusted signal



THE CHALLENGE

Users upload wildly different documents

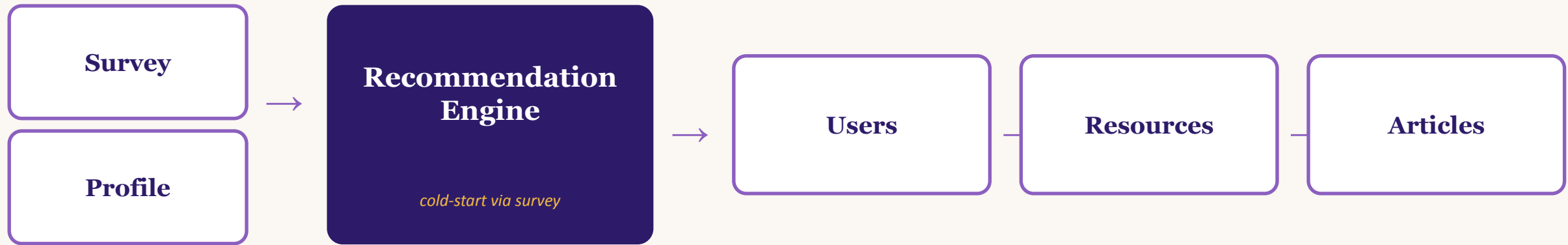
- Hospital PDFs, scans, phone photos
- Different layouts, languages, quality
- OCR errors on every page

MY APPROACH

Preprocessing first, model second

Aggressive text cleaning and normalization made the classifier's job small enough to solve with a lightweight SVM — no GPU required.

Personalized from question one

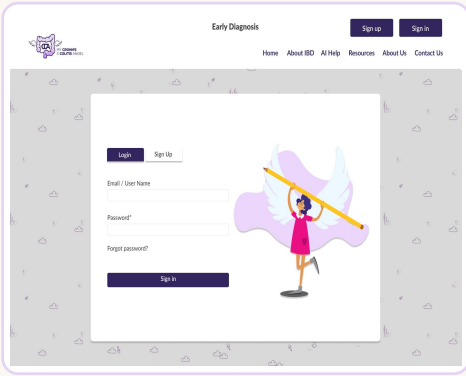


THE PRODUCT PROBLEM • COLD START

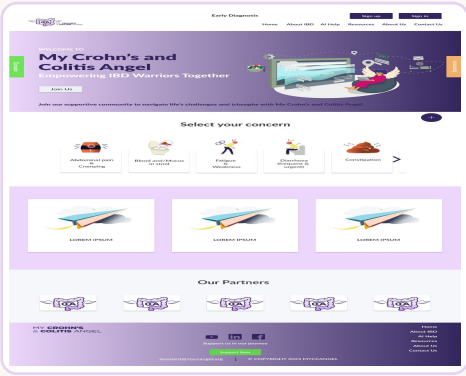
No users, no interactions — how do you recommend?

Answer: the branching survey becomes the cold-start signal. Every intake question maps to feature weights, so day-one recommendations feel personal — long before any behavior data exists.

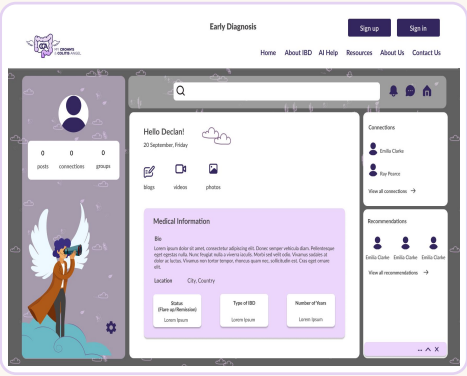
MyCCAngel, screen by screen



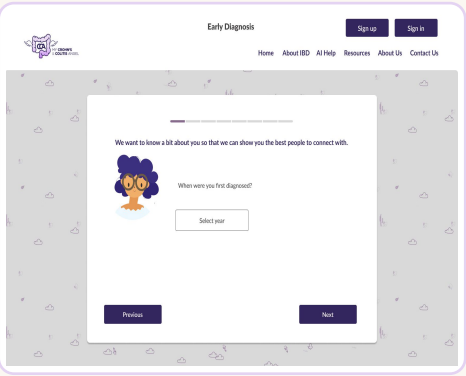
Sign in



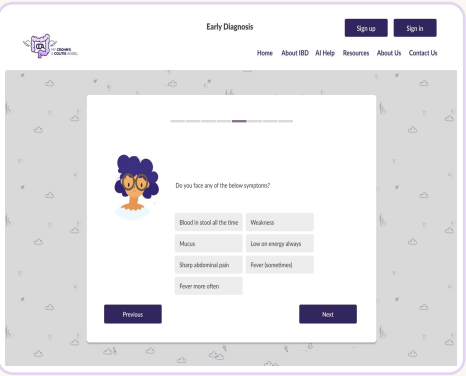
Home



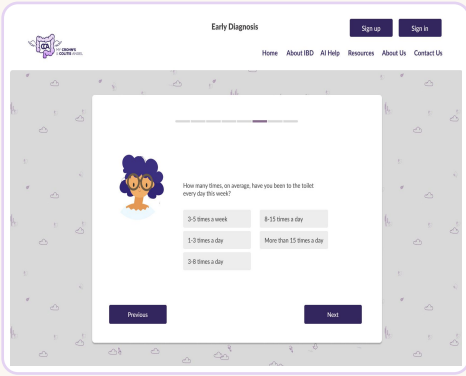
Profile



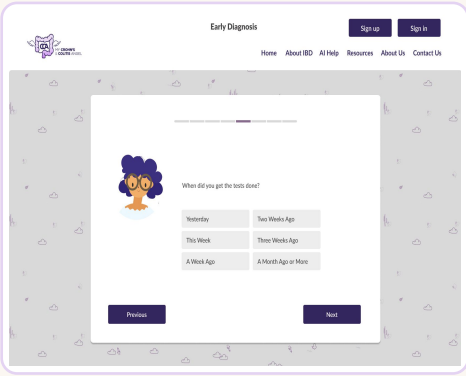
Onboarding



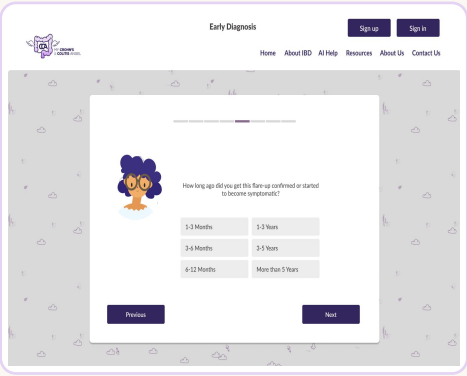
Symptoms



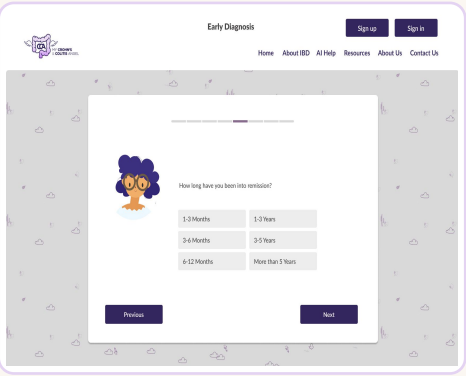
Toilet locator



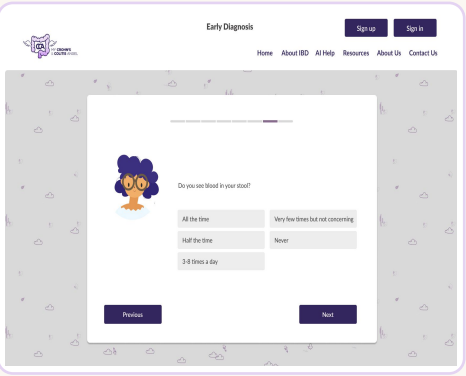
Test tracker



Flare log

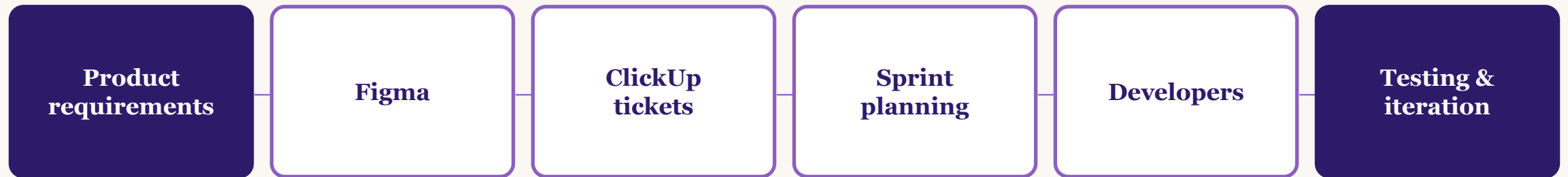


Remission



Alerts

Working with engineering



Requirements	Database schema	Sprints & tasks	QA & iteration
Wrote specs and acceptance criteria for every feature — the source of truth for design and dev.	Designed the data model connecting users, medical records, survey answers, and recommendations.	Ran ClickUp, prioritized the backlog, led standups, unblocked frontend and backend daily.	Tested every build, wrote bug reports, closed the loop back into the next sprint.

As Product Lead, I...

- 1 Conducted user research
- 2 Defined product requirements
- 3 Designed onboarding logic
- 4 Created the database schema
- 5 Collaborated in Figma with UX
- 6 Planned the AI roadmap
- 7 Built the ML verification model
- 8 Designed the recommendation engine
- 9 Managed frontend & backend devs
- 10 Tested, prioritized, iterated

Why we paused before launch

MyCCAngel didn't ship publicly. That's a product decision I stand behind.

Regulatory weight

Medical AI in healthcare needs partnerships and compliance we didn't yet have.

Trust before scale

Launching without clinical validation would have burned the credibility we were building.

Sustainable model

Recommendations are only as good as the content library behind them — we needed publishers on board.

Killing your own launch date is a product skill. Shipping the wrong thing is the real failure.

Where MyCCAngel goes from here

1

Clinical partnerships

One hospital pilot to validate the verification model in-context.

2

Content network

Onboard 2–3 trusted publishers to seed the recommendation library.

3

LLM layer, carefully

Wrap the chatbot with retrieval over verified content only.
No open-ended generation.

4

Outcomes, not usage

Measure flare avoidance and quality-of-life scores — not DAUs.

Five lessons in AI product work

01

Trust is the feature

In healthcare AI, credibility isn't marketing — it's the product itself.

02

Cold start is a product problem

You solve it in the onboarding, not the model.

03

Preprocessing beats parameters

A clean pipeline made a lightweight SVM competitive with heavier models.

04

Prioritization > production

Deciding what NOT to build was the highest-leverage work I did.

05

Story sells strategy

Engineers, designers, and stakeholders align faster around a narrative than a spec.

Building MyCCAngel taught me...

Successful AI products aren't built by starting with AI.

They're built by deeply understanding users, earning trust, and solving meaningful problems — one decision at a time.

Product Strategy

AI Products

Healthcare

Machine Learning

UX

Cross-functional Leadership

How it was built

PRODUCT

User research · JTBD · Prioritization frameworks · Roadmapping

DESIGN

Figma · Design systems · Branching onboarding flows

AI / ML

OCR pipeline · Text normalization · SVM classifier · Recommendation engine

ENGINEERING

Database schema · REST APIs · Frontend & backend coordination

PROCESS

ClickUp · Sprint planning · Daily standups · QA loop